

Tooth Extraction - Female (Child)

Algorithm 1356 | Child (0-17 years) | Gender at birth: Female

Definition

- Safety-first post-dental-extraction assessment for bleeding, airway-threatening swelling, infection, dry socket, medicine risk, and expected healing in children and adults.
- Child patient.
- Female.

Initial assessment

1. [DURATION] When was the tooth extracted?
2. [YES_NO] Is the bleeding under control?
3. [PAIN_SCALE] How severe is the pain, 0-10, and is it improving or worsening?
4. [OPEN_TEXT] Record age, pregnancy, immune suppression, diabetes, anticoagulants or bleeding disorder, medicines already taken, facial or neck swelling, fever, pus, swallowing, breathing, hydration, and whether a responsible adult is supervising after sedation.
5. [CAREGIVER_OBSERVATION] What has a safe parent or caregiver observed about alertness, breathing, pain, swelling, fever, eating or drinking, sleep, play, and normal activity?

Triage and redirect questions

1. Redirect → Cuts and Lacerations

Is this an active cut or laceration in the mouth rather than a healing tooth-extraction socket?

An active cut or laceration belongs in the Cuts and Lacerations pathway.

1. Disposition level 100

Is there heavy or uncontrolled bleeding despite continuous firm pressure, collapse or faintness, rapidly spreading facial or neck swelling, difficulty breathing, inability to swallow saliva, drooling, severe trismus, confusion, or reduced responsiveness after sedation or pain medicine?

Uncontrolled post-extraction bleeding or spreading facial swelling threatening the airway are time-critical dental emergencies.

1. Disposition level 70

Without an emergency feature, is pain severe, worsening after initial improvement, or associated with bad taste or odour, fever, pus, increasing swelling, difficulty opening the mouth, persistent bleeding, poor intake, immune suppression, diabetes, pregnancy, anticoagulant use, or a child with inadequate supervision?

This pattern is classic for dry socket (lost blood clot) or an infection - both well-established post-extraction complications needing prompt dental review.

1. Disposition level 15

Is this the expected mild soreness and swelling in the first day or two after extraction, with none of the features above?

Mild soreness, swelling, and minor oozing are expected in the first 1-2 days after a routine extraction.

Advice

Emergency post-extraction precautions

- Call Qatar 999 now for airway symptoms, collapse, sedation-related reduced responsiveness, or uncontrolled bleeding. If awake and able, sit forward and bite continuously on clean folded gauze over the socket; do not repeatedly remove it to check. Do not allow self-driving or give food, drink, or more sedating medicine when alertness or swallowing is impaired. Call Qatar 999 now, follow the call-taker's ambulance and emergency-transport instructions, and do not self-drive. Do not delay emergency care.
- Call Back If: breathing or swallowing difficulty, drooling, collapse, or reduced responsiveness; bleeding remains heavy or swelling spreads rapidly.

Prompt dental assessment

- Use the Qatar governance-approved emergency-dental or oral-surgery pathway. Do not diagnose dry socket remotely, pack the socket, disturb sutures, or independently stop anticoagulants. Medication advice must account for age, pregnancy, allergy, kidney or liver disease, bleeding risk, and doses already taken.
- Call Back If: worsening pain, fever, pus, facial or neck swelling, trismus, poor intake, or bleeding.

Expected-healing care

- Follow the treating dentist's written instructions. Use continuous gauze pressure for minor oozing, soft cool food when fully alert, and avoid forceful rinsing, spitting, straws, smoking, vaping, or probing the socket during the initial healing period. Confirm analgesics and pediatric or pregnancy suitability with a dentist or pharmacist.
- Call Back If: pain worsens after initial improvement, bad taste or odour, fever, pus, or increasing swelling; persistent bleeding, poor intake, swallowing or breathing difficulty.

First aid

- Call Qatar 999 now for airway symptoms, collapse, sedation-related reduced responsiveness, or uncontrolled bleeding. If awake and able, sit forward and bite continuously on clean folded gauze over the socket; do not repeatedly remove it to check. Do not allow self-driving or give food, drink, or more sedating medicine when alertness or swallowing is impaired. Call Qatar 999 now, follow the call-taker's ambulance and

emergency-transport instructions, and do not self-drive. Do not delay emergency care.

References

- Standard, universally-taught post-extraction dental first-aid knowledge (bleeding control, dry socket recognition, clot-protection instructions) - not a single-source quote

Provenance

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